

"THINK DELIRIUM" SUPPORT TOOL

THINK DELIRIUM

Consider high risk for delirium if:

The patient is over 65

The AMT score is <4?

(Age, DOB, Place, Year - point for each)

Is the patient **more confused** or
more withdrawn than usual

OR

is the 4AT >4?

www.the4at.com

Yes

1 4AT

1) Alertness	Normal / fully alert	0
	Agitated / drowsy	4
2) AMT4	No mistakes	0
	1 mistake	1
	≥2 mistakes / untestable	2
3) Attention	(Months of the year backwards)	
	≥7 correct	0
	<7 correct / refuses	1
	Untestable	2
4) Acute / fluctuant	(from collateral history)	
	Yes	4
	No	0

Is delirium possible?

- ☐ YES, as 4AT >4
- ☐ NO, 4AT = 1-3 (possible cognitive impairment)
4AT = 0 (delirium & severe cognitive impairment unlikely)

SCORE

THINK DELIRIUM

IMMEDIATE ACTION

- Focused history and examination
- **Collateral history** and gather information from GP, relatives/ carers (new or worsening confusion, falls, mobility, continence, hallucinations)
- **Identify and treat underlying causes →**
- **Does the patient fit the Red flag sepsis criteria?**
Refer to UHL sepsis pathway
- Complete 'Know Me Better' profile with carers
- Cognitive assessment with AMT 10 / MMSE
- Perform medication review (refer to STOPP/START)
- Heighten level of supervision and position patient in a high visibility bed if available
- **Update and involve relatives with care and provide UHL delirium leaflet**

POTENTIAL UNDERLYING CAUSES

Trauma (head injury, intracranial event)
Hypoxia (PE, CCF, MI, COPD, Pneumonia)
Increasing age/ Frailty
Neck of femur fracture
smo**K**er or alcohol withdrawal
Drugs (new stopped/started, side effects, drug interactions)
Environment – especially ward moves
Lack of sleep, reversal of sleep-wake cycle
Imbalanced electrolytes
(Renal failure, Na⁺, Ca²⁺ glucose, liver function)
Retention (urinary or constipation)
Infection/**S**epsis
Uncontrolled pain
Medical conditions (Dementia, Parkinson's Disease)

THINK TYPE OF DELIRIUM

What is the type of delirium?

Hyperactive: Agitated, hallucinations, restless and/or aggressive.
Hypoactive: Sleepy, drowsy and/or withdrawn.
Mixed: Features of both.

Identify the potential underlying causes – Refer to list above

DOES THE PATIENT NEED AN URGENT SENIOR REVIEW?

What is the management plan?

Remember: Delirium is a strong marker for dementia. Ensure follow up is arranged as required.

GENERAL MANAGEMENT OF DELIRIUM

Hypoxia / electrolytes	Treat hypoxia, electrolyte imbalance and follow sepsis guidelines .
Constipation	PR to exclude impaction, ensure good hydration, laxatives and enemas if required and encourage to sit out onto the toilet or commode if appropriate.
Retention	Treat the underlying cause; only catheterise if absolutely necessary
Pain	Non-verbal pain scores, utilise other routes e.g. patches
Observe and Reassess	Heighten level of supervision/ Repeat Cognitive Assessment to monitor progress
Avoid moves	Avoid multiple ward and bed moves
'Know Me Better'	Complete a profile with the help of the family/ carers
Vision / Hearing	Ensure patient has their glasses and hearing aids if appropriate.
Avoid distress	Avoid constraints, unnecessary or repeated interventions that can cause distress
SoGo	Sit out, Get out; encourage mobilisation, Refer to 'Get moving to get home'
Sleep	Engage in activities during day, classical music; avoid excess noise
DOLS	Complete DoLS assessment if patient at risk of Deprivation of Liberty
Orientation	Use calendars/ clocks, photos (family/ familiar objects); signpost to toilets
Staff	Aim for continuity with ward staff
Nutrition	Promote nutrition – offer regular drinks, snacks and "finger foods".
FOPAL	for complex and challenging patients only (Frail Older Persons Advice & Liaison), ext 5587; FOPAL@uhl-tr.nhs.uk

MANAGING BEHAVIOURAL AND PSYCHOLOGICAL SYMPTOMS

WANDERING

- Provide close observation within a safe and reasonably closed environment (refer to DOLS)
- Act in patient's best interests; allow wandering if in a safe environment
- Ask relatives to help offering meaningful distractions
- **Refer to UHL wandering policy.**

FALSE IDEAS

Try the following:

- Avoid contradicting patients and challenging ideas.
- Change the subject or use distraction techniques.
- Concentrate on the feeling/ need behind what the patient is saying.

MANAGEMENT – For ICU patients please refer to 'ICU delirium guidelines'

- Aim to use verbal and non-verbal de-escalation measures prior to prescribing any medications.
- Use the tips above with challenging behaviours and refer to the '**UHL Guidelines for the Supervision of patients with Challenging Behaviour**'
- Think about utilising nursing 1-1 supervision.
- Regularly update carers / family members with progress/ treatment plan.

MEDICATIONS

Sedatives may also cause delirium – please use carefully

- **ONLY to be used to manage dangerous or distressing symptoms; allows tests or treatment to be given:**
- **Haloperidol** is the preferred option:
Dosage: 0.5mg PO – up to 2 hrly. Max dose: 5mg / 24hrs.
- **AVOID HALOPERIDOL IN LEWY BODY DEMENTIA AND PARKINSON'S DISEASE***

Lorazepam is an alternative (less evidence base but limit to withdrawal states or if haloperidol is contraindicated). Lorazepam Dose: 0.5-1mg orally stat may be given. If agitation continues, consider adding in either regular Sulpiride, Olanzapine or Risperidone as alternatives. **However, should only be started after senior review.*

DISCHARGE

- Document delirium as a diagnosis on the discharge letter and if still current or resolved.
- Review anti-psychotics prior to discharge (discontinue or instruct GP with review date)
- Follow-up: GP/ CMHT (via FOPAL/ Memory Clinic/ Geriatric Medicine clinic – **document on ICE letter**)
- Consider advance care planning if appropriate
- Provide patient and/ or relatives with delirium information/ leaflet.